Nivolumab, ipilimumab, pemetrexed and cisplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk.
- No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only.
- Continue immune symptom education, bloods each treatment and glucose/thyroid function tests (TFT) each 42-day cycle.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00714 v4 · pNCCP 00714 supportive care; NCIS V8 Table 1 p4 · NCIS low-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required